

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Jan 23, 2026

with Gregory Krause, MD at Mass Eye and Ear Otolaryngology Clinic



Notes from Care Team



Progress Notes by Gregory Krause, MD at 1/23/2026 2:15 PM



Mass General Brigham
Mass Eye and Ear



HARVARD
MEDICAL SCHOOL

MASSACHUSETTS EYE AND EAR INFIRMARY DEPARTMENT--OTOLARYNGOLOGY

Date: 1/23/2026

Patient: Daniel S May DOB: 10/11/1989 MEEI MRN: 8026121

PCP: Pcp, Not Required

Chief Complaint

Patient presents with

- Follow-up

History of Present Illness

The patient is a 36-year-old male who presents for evaluation of left ear infection.

He reports persistent drainage from his left ear, which has not shown any signs of improvement. He was last seen about a month ago. He believes he has an ear infection in the left ear with the tube. He describes a sensation of fullness in the ear, accompanied by mild pain and tinnitus. He also experiences similar symptoms in his right ear, although they are more pronounced in the left. He has been unable to secure an appointment with

an infectious disease specialist due to difficulties in communication. He does not have a follow-up scheduled with Dr. Gray.

He has been experiencing constant nasal congestion, necessitating frequent nose blowing. He describes a sensation of sticky mucus in his nasal cavities, sinuses, eustachian tubes, and ears, which either fails to expel or does so incompletely. He occasionally experiences coughing and feels that the condition is beginning to affect his daily life and work. He has a history of chronic sinus issues and has undergone two nasal/sinus surgeries. He was previously on budesonide rinses, which provided some relief, but had to discontinue them due to financial constraints. He recently started a new job and is considering resuming the budesonide treatment.

PAST SURGICAL HISTORY:

- Sinus surgery, approximately 10 to 15 years ago (two procedures)

EXAMINATION

There were no vitals taken for this visit.

GENERAL APPEARANCE: well developed, well nourished patient in no acute distress.

VOICE: normal

BREATHING: normal, not labored, no stridor

FACE: no lesions

FACIAL STRENGTH: normal and symmetric

AU: EACs without discharge, erythema or swelling. TM intact. No middle ear fluid

Nose: clear

OC/OP: without erythema or exudate. No lesions

Neck: No cervical lymphadenopathy

Procedure: Examination of the Ear Under the Microscope

In order to further assess findings identified on otoscopy, binocular microscopy was performed.

Procedure discussed with patient and verbal consent obtained.

Patient placed under the microscope

AD: Canal clear. TM clear.

AS: Canal with mild swelling. PET in place. Unable to visualize if patent. No discharge. TM opacified with middle ear fluid noted. Unable to suction

thru tube secondary to angle of PE tube
Well tolerated, no bleeding or pain.

Assessment & Plan

1. Left acute otitis media

The left ear appears infected, with the eardrum and ear canal showing signs of swelling and redness. Unable to visualize lumen of tube, but no obvious discharge. Augment and floxin drops.

2. Chronic sinusitis.

Persistent sinus congestion and drainage are affecting the eustachian tube and throat. There is a history of chronic sinus issues and two sinus surgeries over 10 years ago. A referral to an infectious disease specialist has been initiated per Mr. May's request. A prescription for budesonide rinses will be sent to the pharmacy. Follow-up with Dr. Gray for ongoing sinus issues is advised.

Risks, benefits, and alternatives of treatment were discussed. The patient was informed that the left ear infection requires oral antibiotics and eardrops, and the chronic sinusitis may benefit from budesonide rinses. The importance of addressing both sinus and ear issues was emphasized, as nasal/sinus problems can contribute to ear symptoms. The patient was advised to follow up next week if the condition worsens or in a couple of months if improvement is noted.

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